

Cholera in Belfast in 1832 and 1848/49

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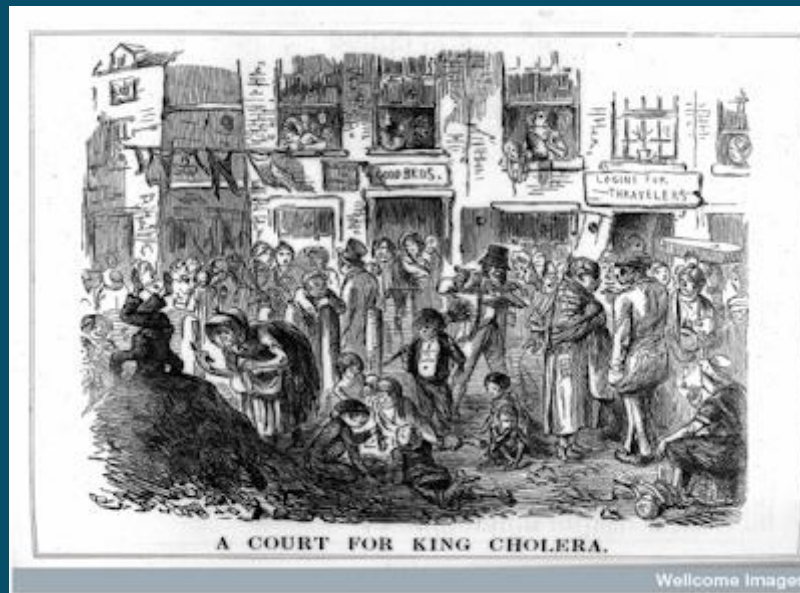
On the 28th of February, 1832 at around midnight, Bernard Murtagh, a 34 year old cooper who resided in a lodging house on Quay Lane Belfast, a narrow street near the River Lagan, became violently ill. Described as a man of irregular habits he had been suffering from diarrhoea for two or three days previously but had not complained of any other symptoms when he went to bed following his usual supper of stirabout and milk. Around midnight his condition worsened and towards morning was accompanied with intense cramps and vomiting, the fluid (from both ends) described as whitish and like milk or meal and water. He was seen by Surgeon McBurney the following morning and was found to be in a state of extreme weakness and collapse, extremely cold and without a perceptible pulse at the wrist. A mustard emetic was administered around midday after which he appeared to revive a little. However, this proved only to be a temporary respite and he died between 7 and 8 p.m. that evening some nineteen hours after becoming ill.



High Street Belfast c.1831. A water cart can be seen to the right of the picture
Source: Ulster Museum IC/High St/831

Murtagh had become the first recognised victim in Ireland to have died from what was then perceived as a new and frightening disease from the East, Asiatic cholera, though in truth it was new only to the West. Cholera, notable for its severity, rapidity and high mortality had been endemic in India in for some time before spreading throughout Asia after 1817 and Europe after 1829. Its signature symptoms, violent vomiting and diarrhoea resembling rice husks were usually accompanied by agonising cramps, muscular spasms, a weakened pulse, low temperature, and a blue tinge to the nails and skin. They were caused by infection with a microorganism, *vibrio cholerae*, usually following the ingestion of water contaminated by the excreta of another cholera sufferer, particularly in places where infected sewage was able to seep into the public water supply. In the towns and cities of nineteenth century Ireland where sanitary practices and sewage systems were often rudimentary at best this particular method of dissemination was a common and deadly hazard.

In Ireland alone around 40% of those who contracted cholera between 1832 and 33 would die as a consequence and in some areas mortality rates were as high as 76%. In a second outbreak during 1848/49 mortality rates were even higher, with the disease finding easy prey in the form of a population severely weakened by Famine and its associated illnesses. Belfast's mortality rate at just 16% was however, much more favourable than anywhere else in the country and was significantly lower than Dublin or Cork who experienced rates in excess of 40%.

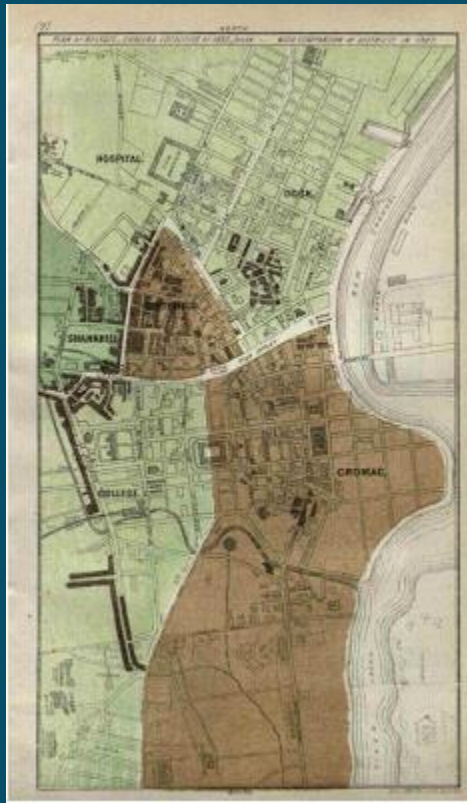


A Court For King Cholera. This famous cartoon depicts conditions conducive to the spread of cholera.

Source: Wellcome Images

Nineteenth century Belfast was Ireland's only industrialised town and outwardly appeared successful and prosperous. Described by one commentator as looking as if it 'had money in its pocket and roast beef for dinner'. However, while industrialisation had created opportunity, it also created serious social issues particularly in the provision of housing, water supply and sanitation. Housing for the labouring poor was laid out in a grid pattern of confined and insanitary courts, lanes and alleys, commonly consisting of two story buildings occupied by two or more families. Few houses were provided with piped water and over 7,000 houses were supplied from public fountains, by water carts, or from pumps sunk by landlords. Sewers were often constructed to deposit their effluent directly into the town's main watercourses and high tides and flooding regularly carried effluvia back onto the streets and into the homes of those who lived in their vicinity, making sanitary conditions and their likelihood of contracting serious illnesses inherently worse.

When cholera came however, Belfast appears to have been as well, if not better prepared to combat the disease than most. The initial response was the remit of the Police Commissioners and of an ad hoc and hastily formed Board of Health. Working closely together, a systematic programme of street cleaning and of whitewashing and fumigating houses was instigated. Temporary hospital accommodation was provided in the grounds of the town's Fever Hospital with Dr Henry McCormac placed in charge. McCormac combined a strict isolation policy with treatments which included bloodletting and the administration of calomel (mercury), opiates and dilute sulphuric acid. Though mortality in the hospital was much higher (22%) than for the rest of the town there does appear to have been less resistance in Belfast to the idea of going to hospital than was the case elsewhere. In Dublin for example, opposition was such that carriages carrying the sick to hospital were occasionally set upon, the patients 'rescued' and the carriages thrown in the Liffey.



Cholera Localities Belfast 1832

Source: A.G. Malcolm 'The Sanitary State of Belfast with Suggestions for its Improvement'
<http://www.tara.tcd.ie>

By the end of the first epidemic over 400 people had died in Belfast and cholera, as did on-going preventative public health provision, passed quickly from public consciousness. Thus, when cholera returned to Ireland in 1848 practically nothing had changed in the way it was fought. However, during this second epidemic, the efforts of Belfast's new Board of Guardians, the physician and sanitary reformer Dr Andrew Malcolm and additional sanitary powers granted to the new Town Corporation by town improvement legislation arguably prevented a much higher death toll than was experienced elsewhere. The Guardians, for example acted in defiance of the Poor Law Commissioners when they opened the Belfast Workhouse in 1841 with ten beds for the reception of the sick, rapidly increasing this to 100. The Corporation introduced new housing regulations and were granted additional sanitary powers, giving them more authority to require landlords and property owners to remove nuisances and pave streets. However, by 1848 Dr Malcolm reported that there continued to be a 'lamentable deficiency' with regard to the removal of offensive remains. As fears of cholera's imminent arrival grew the influential Malcolm rose to the fore to guide the municipal authorities. A Sanitary Committee headed by Malcolm and specifically aimed at dealing with cholera in the first instance was formed in 1848. The Committee published and distributed reports, magistrate's orders were issued for the removal of nuisances, poor families were provided with straw bedding, houses were whitewashed and new sewers were constructed in some parts of the town.

Despite the preparations however, fatalities were almost treble those of 1832. Though Belfast now had two hospitals capable of receiving cholera patients the willingness of the sick to be admitted had declined decidedly. The Committee of the General Hospital attributed the reluctance to 'prejudices or perhaps the state of apathy and hopelessness which accompanies this severe malady' and commented that it was a 'matter of regret, that that the advantages of the hospital were not more generally or duly appreciated by the poor'. By the end of the epidemic 3,538 cases and 1,163 deaths had been recorded but mortality at 33% was again lower than that of other sizable Irish towns. However, in Belfast's worst affected areas, poverty and deficiencies in sanitation and hygiene had clearly been instrumental in the spread of the disease. And while the town's municipal authorities had effected much civic enhancement, major sanitary improvements had not been instigated in the areas of the town where they were most required. Nevertheless, some lasting lessons had been learned and when cholera returned again in 1853 and 1866 mortality rates were almost insignificant by comparison.